

Combination Therapy (Drug A + Methotrexate) in Treating Early Rheumatoid Arthritis

Treating early rheumatoid arthritis (RA) requires managing critical symptoms and promoting the patient's return to their usual quality of life. Current treatment options, including methotrexate (MTX), may not effectively alleviate symptoms. Many patients on MTX show partial improvement but may not respond well and continue to report insufficient relief. This study investigated a combination therapy of Drug A (40 mg every other week) with MTX (20 mg weekly), finding a statistically significant improvement in all patient-reported measures and outperforming the placebo + MTX treatment arm after 24 weeks (Table 1).

Fast and sustained patient-reported relief

The trial used several patient-reported outcomes, including Patient Acceptable Symptom State (PASS) scores, asking if patients considered their current state of health to be acceptable. As early as week 4, nearly 40% of patients on the combination therapy achieved PASS, a change of nearly 16 percentage points compared with the treatment arm on MTX alone ($P < 0.001$) (Figure 1).

Improvements stemming from the combination therapy were sustained into the 24th week, widening to 23 percentage points between treatment arms ($P < 0.001$). The combination therapy offers both rapid and sustained improvements in patient-reported outcomes, specifically providing the relief that patients care about most.

Sustained improvements in patient-reported scores extended across other critical indicators, including fatigue (FACIT-F) and pain (PPA). RA-associated fatigue and pain are often reported as the most significant limiters on a patient's ability to participate in everyday activities. Beyond the physical limitations, these symptoms can have a profound effect on a patient's mood and psychology. Both pain ($P < 0.001$) and fatigue ($P < 0.01$) scores significantly improves in the combination treatment arm over the MTX monotherapy, providing relief for patients who do not respond to MTX (Table 1).

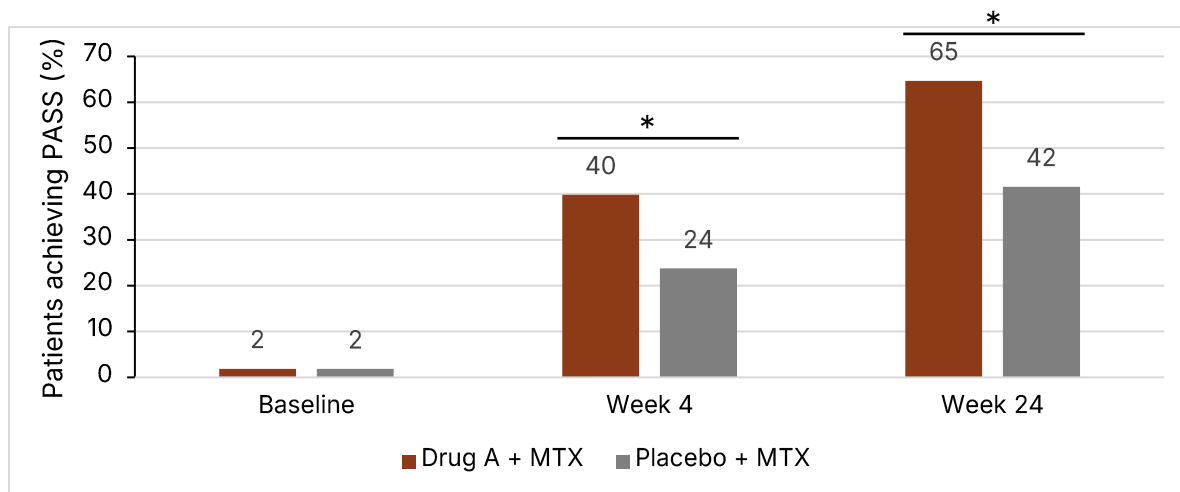


Figure 1. Patients achieving Patient's Acceptable Symptom State (PASS) over 24 weeks. Observed, intention to treat (ITT) population. (* $P < 0.001$)

Return to normal life sooner

From a patient's perspective, many worry about how soon they can return to work and everyday life. Patients on a combination therapy can expect to experience relief from RA-associated symptoms and return to the activities they enjoyed before their diagnosis. Physical function measured by HAQ-DI scores improved by 0.97 points on the combination therapy arm, compared with 0.69 in the MTX monotherapy ($P < 0.001$).

RA-associated pain and fatigue often increase the risk of unemployment, as the patient becomes unable to continue working. Roughly half of the 1,032 patients enrolled in this study were employed at the start of the trial. Among 60 patients from each treatment arm, those treated with MTX alone were nearly three times as likely to become unemployed, compared with those on the combination therapy. The employment and productivity benefits are worth discussing with working-age patients before prescribing. Altogether, patients on the Drug A combination therapy can expect a swift return to work, doing what they love, and a restored quality of life.

What this means for you and your patients

With early RA, establishing a treatment program early and managing symptoms before they progress is critical. Patients on the Drug A combination therapy had statistically significant improvements as early as week 4, and these improvements were sustained well into the 24th week. Prescribers should consider initiating the Drug A combination therapy if patients on MTX alone show no improvement after 4 weeks. During the initial consultation, we suggest using PASS and fatigue scores recommended by the Outcome Measures in Rheumatology (OMERACT) Initiative and following up with patients to determine when adding Drug A to an MTX treatment regimen may be appropriate.

Table 1. Mean change from baseline in patient-reported outcomes by week 24.

Outcome	Drug A + MTX (n=515)	Placebo + MTX (n=517)	Difference ^a	p-value
Physical Function (HAQ-DI) ^b	-0.97	-0.69	-0.28	<0.001
Fatigue (FACIT-F) ^b	11.6	8.78	2.82	0.003
Physician-reported disease activity (PGA) ^b	-40.9	-31.5	-9.4	<0.001
Pain Score (PPA) ^b	-36.8	-27.5	-9.3	<0.001
Quality of life (EQ-5D) ^b	24.9	16.2	8.7	<0.001
Work activity impairment (WPAI) ^c	-31.6	-23.6	-8.0	<0.001
Work Instability (WIS) ^c	-6.45	-5.13	-1.32	0.015

^aDifference between Drug A + MTX and Placebo + MTX. ^bLast observation carried forward (LOCF), intention to treat (ITT) population.

^cObserved, ITT Population. MTX: methotrexate, HAQ-DI: Health Assessment Questionnaire-Disability Index, FACIT-F: Functional Assessment of Chronic Illness Therapy-Fatigue, PGA: Patient's Global Assessment of Disease Activity, PPA: Patient's Pain Assessment, EQ-5D: EuroQol 5-Dimension, WPAI: Work and Activity Impairment, WIS: Work Instability Scale.